Referral Management Policy

MERIDIAN HEALTH PARTNERS
OPERATIONS POLICY MANUAL
SECTION 3: OUTPATIENT REFERRAL MANAGEMENT POLICY
Effective Date: 14 January 2025
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Applies To: All 14 Meridian Health Partners Clinic Sites

3.1 Purpose and Scope 3.1.1 This policy defines the operational standards, routing rules, turnaround time SLAs, and escalation pathways for all outpatient referrals across the 14 multi-specialty clinic sites operated by Meridian Health Partners (MHP). 3.1.2 Compliance with this policy is mandatory for all clinical, administrative, and management personnel processing referral documentation within the MHP Clinical Information System (CIS).

3.2 Specialist Access and Primary Care Sign-Off Requirements 3.2.1 MHP operates an open-access model for select outpatient specialties. Patients may self-refer or book directly without prior primary care sign-off for the following departments: * (a) Department of Physical Therapy and Rehabilitation * (b) Department of Dermatology (Non-Surgical/Cosmetic and General Outpatient) * (c) Department of Nutrition and Dietetics * (d) Department of Behavioral Health and Counseling Services

3.2.2 All other MHP clinical specialties require formal primary care sign-off before an outpatient appointment can be scheduled. Specialties requiring mandatory primary care provider (PCP) authorization include, but are not limited to:

* (a) Department of Cardiology
* (b) Department of Neurology
* (c) Department of Orthopedic Surgery
* (d) Department of Gastroenterology
* (e) Department of Endocrinology
* (f) Department of Oncology and Hematology

3.2.3 Direct self-referrals presented to specialties listed under Clause 3.2.2 shall be rejected by the Central Intake Unit (CIU) within 4 business hours of receipt and redirected to the patient's designated MHP primary care physician for initial assessment.

3.3 Urgency Bands and Turnaround Time SLAs 3.3.1 The MHP referral tracking system strictly enforces time-to-action SLAs based on the clinical urgency band assigned by the referring clinician or triage nurse. 3.3.2 Routine Referrals: Actioned within 5 business days from the date of initial electronic submission into the CIS. 3.3.3 Priority Referrals: Actioned within 2 business days from the date of initial electronic submission into the CIS. 3.3.4 Urgent Referrals: Actioned within 1 business day, that is by the end of the next working day, from the date of initial electronic submission into the CIS. 3.3.5 Emergent Presentations: Emergent presentations are never referred through the outpatient system. Any clinical presentation deemed emergent by any staff member must be directed to emergency care immediately via emergency

medical services (EMS) dispatch or direct transfer to the nearest accredited hospital emergency department. Outpatient referral queues shall reject any submission flagged as emergent.

3.4 SLA Overrides and Authorization Limits 3.4.1 Standard turnaround time SLAs may be overridden only by designated executive clinical leaders when extraordinary operational constraints or individual patient vulnerability demands accelerated intervention. 3.4.2 The following named roles possess sole authority to issue an official SLA override: * (a) Dr. Aris Thorne, Chief Medical Officer * (b) Dr. Brenda Vance, VP of Clinical Operations * (c) Marcus Sterling, Director of Central Intake and Scheduling

3.4.3 All SLA overrides must be documented within the CIS referral audit log, detailing the clinical justification and the identity of the authorizing official within 24 hours of the override action.

3.5 Escalation Ladder for Breached or Stalled Referrals 3.5.1 When a referral tracking ticket exceeds 75% of its allocated SLA window without clinical review or scheduling action, the CIS triggers an automated warning. 3.5.2 If an SLA is breached, the referral escalates automatically through the following named role titles: * (a) Level 1 Escalation (At SLA Breach + 4 Business Hours): Assigned to the Site Referral Coordinator on duty at the destination clinic. * (b) Level 2 Escalation (At SLA Breach + 24 Business Hours): Assigned to Sarah Jenkins, Regional Operations Manager for the respective MHP cluster. * (c) Level 3 Escalation (At SLA Breach + 48 Business Hours): Assigned to Dr. Brenda Vance, VP of Clinical Operations. * (d) Level 4 Escalation (At SLA Breach + 72 Business Hours): Assigned to Dr. Aris Thorne, Chief Medical Officer, for immediate institutional intervention and external network triage if required.

3.5.3 Each escalated referral requires a written root-cause analysis submitted by the Level 2 escalation owner to the MHP Quality and Safety Committee within 5 business days of resolution.

3.6 Documented Exceptions to the Standard Pathway 3.6.1 The standard referral intake, routing, and SLA pathway shall not apply in three specific, documented operational scenarios, as outlined below. 3.6.2 Exception One: Intra-Network Acute Transfers. When an outpatient clinician identifies a deteriorating condition during a scheduled visit that does not meet emergency department transfer criteria but requires same-day specialty evaluation, the provider shall utilize the Direct Intra-Clinic Warm Handover Protocol (Form MHP-PATH-102). Bypassing standard CIU queues, the clinician must contact the destination

specialty charge nurse via Vocera secure voice badge, secure a bed or slot within 60 minutes, and complete the electronic transfer form within 2 hours. 3.6.3 Exception Two: Continuation of Care for Chronically Managed Patients. Patients enrolled in designated MHP Disease Management Programs (specifically Heart Failure, Advanced Diabetes, and Chronic Kidney Disease stages 4–5) who experience an acute exacerbation monitored via remote patient telemetry are exempt from primary care re-authorization. Specialist follow-up orders generated automatically by the remote monitoring clinical decision support engine flow directly to the sub-specialty scheduling queue, bypassing primary-care sign-off, provided an active care plan authored by the attending specialist exists within the preceding 180 calendar days. 3.6.4 Exception Three: Continuity Post-Discharge from MHP Affiliated Inpatient Units. Patients discharged from an inpatient facility where an MHP-employed hospitalist or surgeon directed inpatient care do not require a separate outpatient primary care sign-off for their mandatory post-operative or post-discharge follow-up visit. The discharge coordinator shall book the follow-up appointment directly using the Inpatient Discharge Link module in the CIS during the discharge workflow, overriding standard outpatient referral gates.

Document Control:

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